

ملاحظات :

ختم
شعار الجمهورية

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Name: سماح يوسف احمد البقل National ID: 27207030102741	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



06/11/2026

MDT FORM

Name:	Samah Yousef Ahmed .	MRN:	355
Age:	54 Y	ID:	27207030102741
Menopausal Sta	postmenopausal	Phone:	
NRS SCORE:		Marital Status:	married
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC GII
ER: 0/8 **PR:** 3/8 **HER2:** 3+ **KI67:** 40%

Sonomammography:Mets Workup:History:

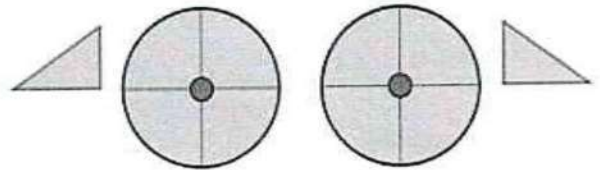
A case of Her2+ BC, cT4b N2 M0...received NACT TCHP x8 (HP only 7 cycles...underwent Rt MRM in 2/6/2026, revealing residual ypT4b N3 M0.

Recommendations:MDT Outcome & Plan

- For:
- 1.IHC on residual disease ER, PR, Her2
 - 2.PORT to RT CW, PCLN, Axilla, IMC
 - 3.Adjuvant Kadcyca x 14
 - 4.Adjuvant ET according to residual IHC

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T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شيماء لاشين	د/ ندى حسن				

Patient Name	: الأستاذ / سماح يوسف احمد البقلي	Request No.	: 13007268
Patient No.	: 27207030102741	Request Date	: 18/10/2025
Path No.	: 3184/25	Verification Date	: 26/10/2025
Sex	: Female	Financial Category	: جوسلاف روسي - لفقة الدولة
Age	: 53 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

IPT:

- ER: Negative (score 0/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).
- Ki67%: 40%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guimei.

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 29/07/2026 09:00:57

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جمهورية مصر العربية
بطاقة تحقيق الشخصية

سماح
يوسف احمد البقلى

ابن المزرعة م نيركو الجديدة الشطر ٣
المعادي - القاهرة

١٩٧٢/٠٧/٢٢ ٢٧٢٠٧٠٣٠١٠٢٧٤١

JU2522176

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٢٠٢٣/٠٧/٢٢ ٢٧٢٠٧٠٣٠١٠٢٧٤١

حاصلة على بكالوريوس تجارة

انثى مسلمة متروجة

المعتمد بالله محمد سعيد محمد كامل

الطاقة سارية حتى ٢٠٢٣/٠٧/٢٢

Patient Name : الأستاذ / سماح يوسف احمد البقلى	Request No. : 13007268
Patient No. : 27207030102741	Request Date : 18/10/2025
Path No. : 3184/25	Verification Date : 26/10/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 53 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right UOQ breast mass.
Nature of Specimen: Tru-cut biopsy, collected on 18/10/2025.

Gross Examination:

- Four fragmented grey white tissue cores, the longest was 0.8 cm. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * DCIS: 25% Intermediate grade ductal carcinoma insitu (Solid subtype).
- * Tumor infiltrating lymphocytes (TILs): 20%.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Negative nuclear reaction in tumor cells (score 0/8).
 - PR: Weak nuclear reaction in 7% of tumor cells (score 3/8).
 - Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
 - Ki67%: 40%.

Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.
- 25% Intermediate grade ductal carcinoma insitu (Solid subtype).

Verified by :

Dr.Duaa Mohamed

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S C A N

Name: Samah Youssef Albakaly

ID: 4404397

Referred by: Prof.

Date: 12/10/2025

Bilateral Digital soft tissue mammography & high resolution breast sonography & Elastography revealed :**Right breast :**

- The hallmark of the study is the presence of mild diffuse skin and parenchymal odema of the breast and architecture distortion. There is an illdefined solid hypoechoic lesion centered in the 10:00 location associated with clustered microcalcifications , measures 32x17 mm in diameters ,hard by Elastography and hypervascular by Doppler.
- Pathologically enlarged right axillary lymph nodes , showing globular appearance , loss of nodal hilar fat .The largest measures 20x21 mm in diameters .

Left breast :

- Normal fibroglandular architecture (ACR A).
- No speculated masses or pathological microcalcification.
- Normal skin thickness & contour .
- No breast solid or cystic masses.Normal tissue elastogram.
- No architecture distortion.
- Small sized likely non-specific axillary lymph nodes.

OPINION :

Right breast UOQ suspicious carcinoma with skin and parenchymal changes as well as ipsilateral axillary lymphadenopathy , **BIRADS 5**, biopsy is recommended.
Essentially normal study of the left breast .**BIRADS 1**.

Kindly accept my best regards,
Prof. Dr. Naglaa Abdel Razek, M.D.

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

Category	Suspected Cancer Risk
BIRADS 0: Non-informative study.	0%
BIRADS 1: Normal study.	0%
BIRADS 2: Benign findings.	2%
BIRADS 3: Probably benign.	50%
BIRADS 4: Probably malignant.	
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant.	98-100%
BIRADS 6: Pathologically proven malignant lesion.	

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Mammogram

[DOCTYPE:MAMMOGRAM]

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OPINION:

- Metabolically active right breast neoplasm with right axillary lymph nodal deposits, for correlation with histopathology.

Radiologists

Dr. Mohamed Shahin, MD
Dr. Mohamed Hoseny, MD

Nuclear Medicine Physicians

Dr. Ahmed Badawy, MD
Dr. Doaa Nabil, MSc



S C A N

Name: Samah Youssef El-Bakaly

ID: 4404397

Date: 21/10/2025

¹⁸F-FDG PET/CT examination

Clinical Indication:

- Patient presented with right breast mass, for assessment.

Administered activity: 333 MBq. ¹⁸F-FDG

B.G.L.: 109 mg/dL

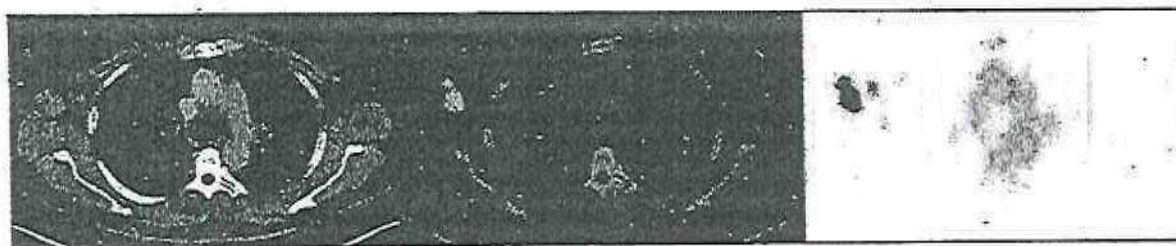
Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction
- Diagnostic multislice CT examination after IV contrast administration of the same region.

1. PET/CT Findings:



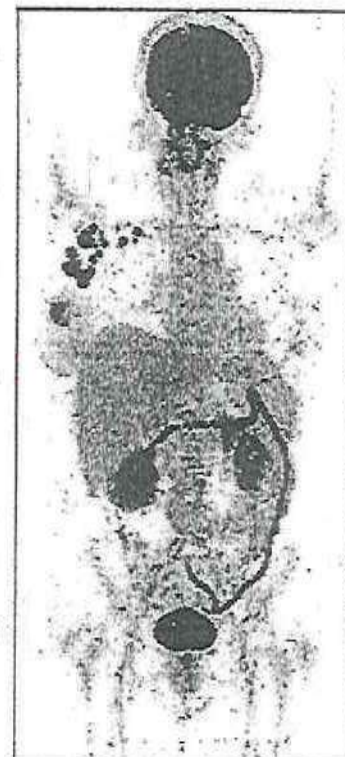
- **Right breast:**
 - Increased FDG uptake within enhancing nodule at upper outer quadrant with SUVmax 9.4 and measures 2.6 x 1.5 cm.
 - Diffuse FDG activity within skin thickening.
 - No skin or chest wall infiltration.
- **Left breast,** normal parenchyma with no focal FDG avid lesions.





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- **Lymph nodes:**
 - **Right axillary:** increased FDG uptake within levels I, II, III and pectoral lymph nodes, level I node with SUVmax 14.9 and measures 2.1 x 2 cm.
 - Otherwise, no FDG avid or pathologically enlarged lymph nodes.
- **Lungs:** few tiny bilateral pulmonary nodules reaching 2.6 mm at right middle lobe nodule with no significant FDG activity, *for follow-up*.
- **Liver:**
 - No focal FDG avid lesions.
 - Hypo-dense lesions at IV and VIII segments largest at segment IV measures 5.4 x 4.5 cm with no significant FDG activity, *likely of cysts*.
 - No dilated biliary radicles. Patent portal vein.
- **Pelvic organs,** no focal FDG avid lesions.
- Low grade FDG uptake within left adrenal nodule with low HU, *likely adenoma*.
- **Bone marrow,** no focal FDG avid lytic or sclerotic lesions.
- **Brain,** no focal FDG avid lesions.



II. Diagnostic CT Findings:

- No pleural, pericardial or peritoneal collections.
- The spleen, pancreas, right adrenal, kidneys, aorta and IVC appear unremarkable.
- *Calcular gall bladder.*

21 Oct, 2025

oussef Albakaly

